Supporting a 4-Year-Old Autistic Child Who Is Aggressive Toward Adults and Peers

A practical guide for parents and carers in the United Kingdom

About this document

This guide is for a parent or carer of a 4-year-old autistic child who hits, bites, kicks, scratches, pushes, or throws things at adults and other children, especially at nursery, preschool, or school. It is written to help you understand what may be driving the behaviour, respond safely in the moment, work effectively with the setting, and build skills over time.

Two things to read first:

1. **This is educational information, not medical, psychological, or legal advice.** Every child is different. Use this guide alongside — not instead of — your child's **GP**, **health visitor**, **community paediatrician**, a **behaviour analyst or Positive Behaviour Support (PBS) practitioner**, an **occupational therapist (OT)**, a **speech and language therapist (SLT)**, a **child psychologist**, and your child's education setting and **local authority (LA)**.
2. **Safety comes first — for your child, for other children, and for staff.** If someone is in immediate danger, get help right away (call **999**). You can work on long-term strategies at the same time as keeping everyone safe today.

A message about your child. Aggression in a 4-year-old autistic child is not "bad behaviour" or "bad parenting." It is almost always a form of communication from a child whose language, sensory processing, and emotional-regulation systems are still developing. That does not make the hitting acceptable, and it does not make it your fault. It means the behaviour has a purpose and a cause — and behaviours with a cause can be understood, reduced, and replaced. This is one of the most treatable challenges in early childhood, and 4 is a great age to intervene.

A note on language. In the UK, clinical services often use "autism spectrum disorder (ASD)," but many autistic people and families prefer **"autism"** or **"autistic"**. This guide uses "autistic child" and "autism," which reflects the preferred language of the National Autistic Society and much of the UK community.

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1. Quick-start: what to do this week

If you are overwhelmed, start here. These are the highest-value first steps.

Priority	Action	Why it matters
1	**See your GP or health visitor** to rule out pain, illness, sleep problems, constipation, dental issues, ear infections, reflux, and medication side effects.	A child who is in pain or exhausted cannot regulate. This is the most commonly missed cause.
2	**Ask the setting for an assessment of your child's special educational needs (SEN Support)** and a written **behaviour support plan**.	Under the SEND Code of Practice, the setting must identify and support SEN. This is the backbone of support.
3	**Start an ABC log** (see Section 4).	Patterns you can see in data are patterns you can fix. Guessing rarely works.
4	**Ask the setting for a daily communication log** and a named contact (usually the **SENCO**).	You need to know what is happening each day to respond consistently.
5	**Create a calm-down space** at home and ask the setting to provide one too.	A safe place to regulate prevents escalation and protects others.
6	**Ask your GP to refer you** to the **community paediatrician / neurodevelopmental pathway**, and ask about **occupational therapy (OT)** and **speech and language therapy (SLT)**.	These are the services most directly relevant to this challenge. Waiting lists can be long, so ask early.
7	**Find your local authority's Local Offer and your SENDIASS** (see Section 22).	These are free, statutory sources of local support and advice.

Do not wait for a diagnosis to be "complete" or for a formal assessment to finish before starting steps 1, 3, 4, 5, and 6. A diagnosis is not required for a setting to put SEN Support in place — the setting must support your child based on their needs. Early intervention matters, and 4-year-olds respond well to it.

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2. Understanding why the aggression is happening

Behaviour almost never happens "for no reason." For young autistic children, aggression typically serves one or more of **four functions**. Identifying the function is the single most important step, because the correct response depends entirely on it.

The four common functions

Function	What it looks like	What the child may be "saying"
Escape / avoidance	Hitting when asked to do something (transition, tidy up, sit down, share, work task)	"I can't do this." "This is too hard." "Stop making me."
Access to a tangible	Biting or pushing when a toy is taken or when they want something another child has	"I want that." "Give it back."
Attention	Aggression that reliably brings an adult running, or that happens when adults are distracted	"Look at me." "Talk to me." "Play with me."
Sensory / automatic	Biting own hand, head-banging, repetitive aggression that happens whether or not anyone reacts	"I need this input." "I'm overwhelmed and this helps."

Many incidents serve more than one function, and the function can change over time or differ between home and the setting. That is normal — and it is exactly why you collect data.

Why a 4-year-old autistic child is at higher risk for aggression

Understanding the "why behind the why" helps you respond with compassion and precision.

- **Communication delay.** If a child cannot say "I'm frustrated," "I need a break," "that's mine," or "that's too loud," the body often does the talking. Aggression is efficient — it works fast.
- **Sensory overload.** Nurseries and classrooms are loud, bright, busy, and unpredictable. A child who is already at their sensory limit has almost no capacity left for a demand or a conflict.
- **Difficulty with transitions and change.** Moving from one activity to another, unexpected schedule changes, and being told "no" are common triggers.
- **Delayed emotional regulation.** The part of the brain that manages big feelings is still under construction at 4 — and often develops later in autism. A 4-year-old may have the emotional regulation of a much younger child.
- **Difficulty reading social cues and understanding consequences.** The child may not understand that hitting hurts, or may not connect the action to an outcome.
- **Unmet medical or physical needs.** Pain, hunger, tiredness, constipation, and illness dramatically lower the threshold for aggression (see Section 3).
- **Reinforcement history.** If aggression has ever "worked" — ended a demand, got a toy, brought attention — it has been strengthened. This is not the child being manipulative; it is basic learning.

Reframing: the goal is not punishment

Punishment (time-out, taking away toys, shouting) often *increases* aggression in young autistic children because it adds stress without teaching a replacement skill. The goal is to:

1. **Make the environment easier** so aggression is less likely (prevention).
2. **Teach a replacement skill** that works better than aggression (communication).
3. **Respond consistently** so aggression no longer gets the old payoff (consequence).

This approach is the foundation of **Positive Behaviour Support (PBS)**, the framework recommended across UK health, social care, and education services. PBS focuses on improving quality of life and reducing the need for restrictive intervention.

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3. Rule out medical and physical causes first

Before assuming a behaviour is purely psychological, rule out the physical causes. A child who cannot tell you "my ear hurts" or "my stomach hurts" may hit instead.

Ask your GP or health visitor to consider:

- **Ear infections** — very common, easy to miss, and often painful without obvious signs. Ask about glue ear, which affects hearing and can be missed for months.
- **Dental problems** — teething, tooth decay, abscesses, gum pain. NHS dental care is free for under-18s.
- **Constipation and gut issues** — extremely common in autism and strongly linked to irritability and aggression. Ask specifically about stool frequency, straining, and withholding. NICE guidance covers constipation in children.
- **Reflux (GORD)** — causes pain, especially lying down or after eating.
- **Sleep problems** — obstructive sleep apnoea, restless sleep, difficulty falling asleep. Sleep deprivation alone can cause severe daytime aggression. Ask about a sleep study or referral to a sleep service if snoring or breathing pauses are present.
- **Headaches / migraines.**
- **Skin issues** — eczema, rashes, itching.
- **Allergies and food intolerances** — including reactions that cause discomfort.
- **Epilepsy, including "silent" (absence) seizures or night-time seizures** — these can cause sudden behaviour changes. Ask whether an EEG or paediatric neurology referral is warranted if behaviour changed suddenly or is atypical.
- **Medication side effects** — some medicines used for sleep, allergies, or behaviour can increase irritability. Review *everything* your child takes, including over-the-counter medicines and supplements.
- **Hormonal or metabolic issues** — rare, but worth ruling out if behaviour changed suddenly.
- **Pain anywhere** the child cannot localise or describe.

Practical tips:

- Keep a simple health diary for 2–4 weeks: sleep hours, bowel movements, appetite, illness signs, and behaviour severity. Bring it to appointments.
- Ask: *"Could anything physical be making my child more irritable or aggressive? What should we test for?"*
- Get a **vision and hearing check** — undetected problems add daily frustration. Ask your GP about referral to audiology or orthoptics.
- If behaviour changed suddenly or dramatically, treat that as a medical red flag and seek prompt assessment.

- **Note on waiting times:** NHS neurodevelopmental assessment waits can be long. Ask your GP what the local pathway and likely wait is, and ask about support you can access in the meantime.

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4. The ABC approach: becoming a behaviour detective

The **ABC** method is the core tool for understanding aggression. Every incident has three parts:

- **A — Antecedent:** What happened *right before*? (demand, transition, "no," noise, another child taking a toy, being told to wait, being tired or hungry)
- **B — Behaviour:** What exactly did the child do? (hit a peer with an open hand, bit an adult's arm, threw a chair) — be specific and objective.
- **C — Consequence:** What happened *right after*? (task was removed, adult reprimanded, child got the toy, child was taken to a quiet room, peers moved away)

The consequence tells you the **function**. If the demand stopped after the hit, the function was likely escape. If the toy was returned, it was access. If an adult came over and talked, it was attention. If nothing changed and it happens repeatedly alone, it may be sensory.

How to collect ABC data

Keep it simple and sustainable. A notebook or phone note works. Log the incident as soon as you can, and ask the setting to do the same. Aim to capture **10–20 incidents** before drawing conclusions.

Sample ABC log entry:

Date/time	Setting	Antecedent (A)	Behaviour (B)	Consequence (C)	Possible function
Mon 9:20	Carpet time	Staff said "sit down" after child was already upset by noise	Hit teaching assistant on arm twice	Child removed from the group to the corridor	Escape
Mon 11:05	Free play	Peer took the red car	Bit peer's hand	Peer dropped car; child got car	Access (tangible)
Tue 1:30	Snack	Adult turned away to help another child	Threw cup, screamed	Adult returned attention to child	Attention

Reading the patterns

After 10–20 entries, look for:

- **When** does it cluster? (time of day, after lunch, at transitions, when tired)
- **Where** does it happen? (loud rooms, crowded spaces, unstructured time)
- **With whom?** (specific staff, specific peers)
- **After what demand or event?**
- **What reliably stops it?**

These patterns become the targets of your prevention plan and the setting's behaviour support plan.

A note on "functions" and testing

You do not need to be certain about the function before acting. Start with the most likely function, apply the matching strategy, and watch whether the behaviour decreases. If it does not, revisit your hypothesis. A **behaviour analyst** or a **PBS practitioner** can do a more formal **functional behaviour assessment** if the patterns are unclear. In UK education settings, this is often simply called a behaviour assessment or part of a behaviour support plan.

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5. Safety first: protecting everyone in the moment

Safety planning is not optional — it is the foundation. The goal is to prevent injury while you work on the longer-term plan.

Principles

- **Keep the child and others safe without shaming the child.** The child is not the enemy; the behaviour is the target.
- **Reduce the audience.** Aggression is more likely and harder to stop when peers are watching. Move to a calmer, more private space when safe to do so.
- **Protect the head, face, and neck.** Teach staff and family to keep distance, turn the body, and use barriers (a cushion, a table) rather than grabbing.
- **Do not restrain unless trained and it is truly necessary for immediate safety.** Untrained holds escalate and can injure both child and adult. See Section 13.
- **Clear the area** of objects that can be thrown or used to hit.
- **Keep a phone accessible** in case you need help.

Protective positioning for adults

- Stay at an angle, not directly in front.
- Keep arms relaxed and hands free; do not loom over the child.
- Maintain enough distance to avoid a strike while staying available.
- Use a calm, low, slow voice — volume and speed escalate children.
- Avoid sudden movements and grabbing.

Home safety checklist

- [ ] Safe, soft **calm-down space** with no hard edges or throwable objects.
- [ ] Breakable or heavy items removed from areas where incidents occur.
- [ ] A door or gate that lets you create a safe boundary if needed.
- [ ] A written plan for what to do if the child bites or hits during a nappy change, bath, or car journey.
- [ ] Car safety plan: pull over safely before responding to aggression in the car.
- [ ] Identify one adult who can be called for backup.

Setting safety expectations

Ask the setting for a written plan that includes supervision levels, a safe space, a named team, de-escalation steps, and incident reporting. Details are in Section 12.

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6. De-escalation: what to do during an incident

During an incident, your job is **safety and calm**, not teaching or discipline. Teaching happens afterwards, when everyone is regulated. Trying to reason with, lecture, or punish a child mid-meltdown usually makes it worse — the thinking part of the brain is offline.

The escalation curve

Most incidents move through stages. Matching your response to the stage prevents you from making it worse.

1. **Trigger / early signs** (fidgeting, pacing, whining, flushed face, repeating phrases, covering ears). → *Prevent: reduce demands, offer a break, remove the trigger.*
2. **Agitation / escalation** (louder, red-faced, pushing things, threats of hitting). → *De-escalate: fewer words, more space, offer choices, remove the audience.*
3. **Crisis / peak** (hitting, biting, throwing). → *Safety only: protect people, minimal words, do not discipline, do not crowd.*
4. **De-escalation** (crying, deflating, exhausted). → *Give space and quiet. Do not revisit the incident yet.*
5. **Recovery** (calm, tired, possibly clingy). → *Reconnect, then later teach and problem-solve.*

What to say and do during a crisis

Do:

- Speak in a calm, quiet, slow voice. Fewer words are better.
- Give space. Back up rather than moving in.
- Use short, concrete phrases: *"I'm safe. You're safe." "I'm here." "Let's go to the quiet corner."*
- Offer a simple, low-demand choice: *"Sit here or sit there?"*
- Remove other children from the area calmly and without drama.
- Protect yourself and others with distance and barriers.
- Wait it out if it is safe to do so. The peak passes.

Do not:

- Shout, threaten, or lecture.
- Stand over the child, block them with your body, or grab them.
- Take away a comfort object mid-crisis to punish.
- Ask "why did you do that?" or demand an apology during the crisis.
- Make eye-contact battles or corner the child.
- Discuss the incident with other adults in front of the child in a blaming way.

After the peak (recovery)

- Give 10–20 minutes of low-demand, quiet time.
- Offer water, a snack, a comfort item, or a preferred calming activity.
- Reconnect warmly. The child needs to know they are still loved and safe.
- **Do not** immediately re-teach or punish. Wait until fully calm — often much later or the next day.
- Log the incident (ABC data) while it is fresh.

Teaching later, not during

When the child is calm, you can:

- Use **social stories** or simple picture sequences about what to do instead.
- Role-play the replacement skill (e.g., "hands down, say 'break'").
- Practise the calm-down routine when calm, so it is automatic when stressed.

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7. Building communication so aggression becomes unnecessary

If aggression is communication, the most powerful long-term strategy is to give the child **better, easier ways to communicate** — ideally before they need to escalate. This is called **functional communication training (FCT)**.

Teach a replacement for each function

Function	Teach the child to...	Make it easy
Escape / avoidance	Say or show **"break"** or **"help"**	A "break" card, a sign, a button, or a picture
Access to a tangible	Say or show **"I want ___"** / **"my turn"**	Picture cards, PECS, a communication app
Attention	Say or show **"play with me"** / tap a shoulder	A card or gesture that reliably gets a warm response
Sensory	Use a **sensory tool** or ask for it	Chewy items, a squeeze, a movement break

Key rule: the replacement must work *better and faster* than aggression. If saying "break" does not actually get a break, the child will go back to hitting. Honour the communication every time it is used, especially at first.

Tools that help a 4-year-old communicate

- **Picture exchange (PECS) or picture cards** — the child hands you a card to make a request. Very effective at this age and widely used in the UK.
- **AAC (augmentative and alternative communication)** — a speech-generating app or device. Research shows AAC *does not* hinder speech; it often helps it. Ask your NHS **speech and language therapist (SLT)**.

- **Sign language / gestures** — simple signs for "more," "stop," "break," "help." Makaton is widely used in UK early years settings.
- **Core word boards** — "go," "stop," "more," "help," "want," "no."
- **Now and Next board** — a two-part visual showing what is happening now and next.
- **Choice boards** — two pictures so the child can choose instead of protesting.

Ask your **SLT** to build a communication plan that includes a reliable way to request a break and to say "no" or "stop." Make sure the same system is used at home and in the setting.

Reduce the need to escalate: make requests easy to succeed

- Give **one instruction at a time**, with the child's name first.
- Use **"first-then"**: *"First shoes, then outside."* Pair with pictures.
- Offer **choices within limits**: *"Do you want the red shoes or the blue shoes?"* — the task still happens, but the child has control.
- **Prime** before transitions: *"In two minutes we tidy up."* Use a visual timer.
- Use **visual schedules** so the child knows what is coming next.
- Break tasks into small steps and reinforce each step.

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8. Sensory and regulation strategies

Many 4-year-old autistic children are either **over-responsive** (easily overwhelmed by sound, light, touch, crowds) or **under-responsive / sensory-seeking** (craving movement and deep pressure). Both can drive aggression. An **occupational therapist (OT)** can assess your child and design a "sensory diet."

Signs of sensory overload

- Covering ears, squinting, turning away.
- Increased movement, spinning, crashing.
- Meltdowns in noisy or crowded places.
- Refusing certain textures, foods, clothing, or touch.
- Aggression that spikes in busy, loud, or bright environments.

Calming (down-regulating) strategies

- **Deep pressure:** firm hugs (if the child accepts them), weighted lap pad or blanket (only with OT guidance and age-appropriate weights), rolling a therapy ball gently over the back.
- **Heavy work:** carrying a laundry basket, pushing a pushchair, wall pushes, animal walks, helping carry shopping.
- **Slow, rhythmic movement:** rocking, swinging gently, slow bouncing.
- **Reduced sensory input:** dim lights, quiet space, ear defenders, a hood or hat.
- **Oral input:** chewy tube, crunchy snack, drinking through a straw.
- **Breathing / calm routine:** a simple, practised routine — count to three, blow out "candles," squeeze and release hands.

Alerting / organising strategies (for under-responsive children)

- Movement breaks, jumping, climbing, swinging.
- Fast, rhythmic music and movement.
- Crunchy or cold foods.
- Fidget tools.

Build a "sensory diet" into the day

A sensory diet is a planned schedule of sensory activities across the day to keep the child regulated *before* they tip over. Example:

Time	Activity
Morning before the setting	Heavy work (carry bag), wall pushes
Mid-morning	Movement break, drink through straw
Before transitions	Deep pressure, visual timer, warning
After the setting	Swing, trampoline, quiet time
Before bedtime	Bath, deep pressure, dim lights

Ask the OT to write this plan and ask the setting to follow it. In the UK, OT may be accessed through NHS community services, the setting, or (sometimes) via an EHCP.

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9. Home strategies that reduce aggression

Consistency between home and the setting is one of the strongest predictors of success.

Build predictability

- **Visual schedule** for morning, after the setting, and bedtime. Review it together.
- **Warn before transitions** ("two more minutes," then "one more minute").
- **Same routines, same order, same words.** Predictability lowers anxiety.
- **Prepare for changes** with a social story or a simple explanation in advance.

Make your responses consistent and calm

- Decide in advance how you will respond to hitting (a short, calm, scripted response), and use the same words every time.
- Keep your own voice and body calm. Your regulation helps regulate your child.
- Avoid power struggles. Offer choices instead of ultimatums.
- **Praise and reinforce the behaviour you want**, immediately and often. Catch your child being gentle, using words, waiting, or calming down — and name it: *"You used your words! Great job asking for a break."*
- Use a simple reward system (sticker, token, preferred activity) for gentle hands and using words. Keep it immediate and visual.

Create a calm-down space

- A small corner or tent with soft items, a beanbag, ear defenders, a favourite comfort object, and visuals showing the calm-down steps.
- **Frame it as a positive place, not a punishment.** "The cosy corner is where we go when our body needs to settle." Never send the child there angrily as a penalty.
- Practise going there when calm so it is familiar.
- Ask the setting to provide an equivalent "safe space" or "regulation station."

Reduce known triggers

- Identify triggers from your ABC log and reduce or prepare for them.
- Avoid unnecessary demands when the child is already tired, hungry, or overstimulated.
- Schedule demanding tasks during the child's best-regulated times.
- Keep transitions short and supported.

Teach and practise skills when calm

- **Social stories** — short, simple stories with pictures about hitting, sharing, waiting, and what to do instead.
- **Role-play** — practise "hands down," "say break," "ask for a turn."
- **Model** the behaviour you want and narrate it.
- **Comic strip conversations / picture sequences** for what happened and what to do next time.
- Approaches developed and widely used in the UK that may help: **Intensive Interaction**, **Attention Autism (Gina Davies)**, **SCERTS**, and **TEACCH**. Ask your SLT or educational psychologist about what suits your child.

Specific behaviour-by-behaviour responses

Behaviour	In-the-moment response	What to teach
Hitting	Block safely, minimal words, create distance, protect others	"Hands down" + ask for break/help
Biting	Protect the skin, separate, do not yank the child	Chewy tool + "I need space"
Kicking	Move out of range, lower your body, do not grab legs	"Stop" + request a break
Throwing	Clear throwable objects, do not chase the object	"I want ___" + ask for help
Spitting	Turn away, stay neutral	Drink through straw + "no thank you"
Scratching	Cover arms, create distance	"Stop" card + ask for help

10. Working with the setting: SEN Support, SENCO, and the graduated approach

In England, the **SEND Code of Practice (2015)** sets out how early years settings, schools, and local authorities must support children with special educational needs and disabilities (SEND). (Scotland, Wales, and Northern Ireland have their own frameworks — see the note at the end of this section.)

Key people and terms

- **SENCO** — Special Educational Needs Coordinator. Every early years setting and school must have one. This is your main point of contact.
- **SEN Support** — the support given to a child with SEN in a mainstream setting, without an EHCP.
- **Graduated approach** — a four-part cycle: **Assess, Plan, Do, Review**. This is the statutory way settings should identify and meet needs.
- **Early Years SENCO / Area SENCO** — support for nurseries, preschools, and childminders.
- **Individual Healthcare Plan (IHP)** — a written plan for a child's medical needs in the setting.
- **Behaviour policy** — every school must have one; ask to see it.

The graduated approach in practice

1. **Assess** — the setting gathers information about your child's needs, including your views and any reports from health or other professionals.
2. **Plan** — the setting agrees with you what outcomes to aim for and what support to put in place (this may include a **behaviour support plan** or a **one-page profile**).
3. **Do** — the setting puts the plan into action, with a named person responsible.
4. **Review** — you and the setting review progress on a set date and adjust the plan.

Ask for these meetings to be recorded, and ask for a copy of the plan.

How to request SEN Support

- **Put it in writing** (email is fine) and keep a copy. Example:

> Dear [SENCO / Manager],
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> I am writing about my child, [name], who attends [setting]. [Name] is autistic and is currently hitting and biting adults and other children, which is affecting their safety, wellbeing, and learning. I am asking the setting to put SEN Support in place under the SEND Code of Practice, following the graduated approach (Assess, Plan, Do, Review). I would like a written behaviour support plan and a safety plan, and I would like to meet to discuss this. I am also asking for a daily communication log and a named contact.
>
> Please confirm the date of the meeting and share the plan with me.
>
> Thank you, [Your name]

What to ask the setting for

- A **named SENCO** and regular contact.
- A written **behaviour support plan** based on an assessment of the behaviour's function (see Section 12).
- Staff training on your child's plan and on de-escalation.
- A consistent **daily communication log**.
- A **safe space** for regulation.
- **Adjustments**: visual schedule, transition warnings, sensory breaks, quiet space, communication supports, reduced demands during escalation.
- **Services**: speech and language therapy, occupational therapy, and behaviour support (some of these are health services; ask about how they are accessed locally).
- **Supervision** appropriate to safety needs (this may include additional adult support).
- A clear **crisis protocol** and **incident reporting** so you know what happened.
- Agreement that **the same communication system** (e.g., break card, PECS) is used at home and in the setting.

Partnership mindset

- Approach staff as **partners**, not adversaries — you all want the same outcome.
- Share your ABC data and health information with the team.
- Ask for data on progress, and review it together regularly.
- If you feel the setting is not responding, escalate in writing, request a meeting with the SENCO and the head or manager, and consider contacting **SENDIASS** or **IPSEA** for free advice (see Section 22).

Scotland, Wales, and Northern Ireland

- **Scotland:** uses the **Additional Support for Learning (ASL)** framework, with **Additional Support Needs (ASN)**, **Individualised Educational Programmes (IEPs)**, and **Co-ordinated Support Plans (CSPs)**. Support is via the school and the local authority; **Enquire** is the national advice service.
- **Wales:** uses **Additional Learning Needs (ALN)** under the **Additional Learning Needs and Education Tribunal (Wales) Act 2018**, with **Individual Development Plans (IDPs)**. **SNAP Cymru** provides advice.
- **Northern Ireland:** uses the **Special Educational Needs and Disability (SEND) Act (NI) 2016**, with **Statements of SEN** and **Learning Support Coordinators (LSCs)**. The **Special Educational Needs Advice Centre (SENAC)** provides advice.

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11. Education, Health and Care (EHC) plans and the Local Offer

If SEN Support is not enough to meet your child's needs, the next step is an **Education, Health and Care (EHC) needs assessment**, which may lead to an **EHC plan (EHCP)**.

What an EHCP is

- A **legal document** for children and young people aged 0–25 in England with more complex needs.
- It sets out your child's **special educational needs**, the **outcomes** to be achieved, and the **provision** (support) required, including health and social care.
- It **names a setting** (nursery, school, or other provision) that must admit your child.
- It is **legally enforceable** — the provision must be delivered.

How to request an EHC needs assessment

- You, the setting, or a professional can ask the **local authority (LA)** to carry out an EHC needs assessment. You do not need a diagnosis.
- **Put it in writing** and keep a copy. Example:

> Dear [Local Authority SEND Team],
>
> I am requesting an EHC needs assessment for my child, [name], aged [age], who lives in [area] and attends [setting]. [Name] is autistic and their aggression towards adults and other children is significantly affecting their safety and learning. I believe the setting cannot meet their needs through SEN Support alone. I attach evidence including [reports, ABC data, health information].
>
> Please confirm receipt and the timeline for your decision.
>
> Thank you, [Your name]

- **Key timelines (England):** the LA must tell you within **6 weeks** whether it will assess; if it will, the whole process to a final EHCP should take no more than **20 weeks** (excluding exceptions). If the LA refuses to assess or to issue a plan, you have the right to **appeal to the First-tier Tribunal (Special Educational Needs and Disability)**.
- **You can appeal** decisions about whether to assess, whether to issue a plan, the contents of the plan, and the named setting. Appeals usually require **mediation** first. Get advice from **SENDIASS** or **IPSEA**.

The Local Offer

- Every local authority in England must publish a **Local Offer** — a directory of the services and support available locally for children and young people with SEND.
- Find it on your council's website. It should cover education, health, and social care, including short breaks, support groups, and specialist services.
- Ask your SENDIASS or LA if you cannot find it.

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12. What to ask for in a behaviour support and safety plan

A good plan is specific, written, and followed by everyone. Use this as a checklist.

Prevention - [ ] Identified triggers and how to reduce them. - [ ] Visual schedule and transition warnings. - [ ] Sensory supports and scheduled breaks. - [ ] Communication system available at all times. - [ ] Predictable routines and clear expectations.

Teaching - [ ] Replacement behaviours defined (e.g., request break, ask for help). - [ ] How and when these are taught and practised. - [ ] Social stories / role-play included.

Response - [ ] Step-by-step de-escalation script for staff. - [ ] Who removes other children, who supports your child. - [ ] When and how to move to a safe space. - [ ] Explicit rule: **restrictive physical intervention only as a last resort for immediate safety, by trained staff, and recorded** (see Section 13).

Crisis - [ ] Named crisis team and backup. - [ ] When to call you; when to call emergency services. - [ ] How injuries are documented and reported to you. - [ ] Post-incident debrief and reconnection plan.

Communication - [ ] Daily log (behaviour, triggers, strategies used, successes). - [ ] Regular team meetings. - [ ] Immediate notification of any injury or restrictive intervention.

Review - [ ] Data reviewed on a set schedule (e.g., every 4–6 weeks). - [ ] Plan updated as the child grows and skills change.

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13. Restrictive intervention, reasonable force, and your rights

Because your child's aggression may lead to physical intervention at the setting, you need to know the rules.

Key concepts

- **Reasonable force:** school staff have a legal power to use **reasonable force** to prevent a pupil from committing an offence, injuring themselves or others, or damaging property, and to maintain good order and discipline. The force must be **proportionate** and the **minimum necessary**. It is governed by **Section 93 of the Education and Inspections Act 2006** and DfE guidance (**"Use of reasonable force," 2013**).
- **Restrictive physical intervention / restraint:** physically holding or restricting a child. In health and social care, this is governed by the **Mental Capacity Act 2005** and **Deprivation of Liberty Safeguards**, and should follow **Positive Behaviour Support** principles and be a **last resort**.
- **Seclusion:** isolating a child alone in a room they cannot leave. This is **highly controversial**, and DfE guidance says it should only ever be used as a last resort, for the shortest possible time, with constant supervision, and be recorded.
- **Reducing the need for restrictive intervention:** UK policy (including the **Positive and Proactive Care** guidance, 2014) expects services to **reduce** the use of restrictive intervention, not increase it.

What good practice looks like

- Restrictive intervention is used only for **immediate safety**, by **trained staff**, as the **least restrictive option**, for the **shortest time**.
- It is **never** used as punishment, for convenience, or for staff shortages.
- It is **recorded** in writing and **reported to parents/carers the same day**.
- The child is **monitored** throughout for distress or medical risk.

- Staff are trained in **de-escalation** and in safe, approved holds.
- The **behaviour support plan** aims to make restrictive intervention unnecessary over time.

Questions to ask the setting

- What is your written policy on the use of reasonable force and restrictive intervention?
- Which staff are trained, and in what method?
- Will you notify me the same day if physical intervention or seclusion is used?
- How will you prevent the need for it (de-escalation, safe space, staffing)?
- What is your policy on physical intervention with a 4-year-old?

If you believe your child is being restrained or secluded inappropriately

- Document everything: dates, times, who was present, what happened, any marks or injuries (photograph them).
- Request the written incident records.
- Request a meeting with the SENCO and head or manager, and put your concerns in writing.
- Ask for the **behaviour support plan** to be reviewed by a behaviour specialist.
- Get free advice from **SENDIASS**, **IPSEA**, or the **National Autistic Society**.
- You can **complain** to the setting's governing body, to the **local authority**, and in serious cases to the **Local Government and Social Care Ombudsman** (for LA and some school matters) or the **DfE**.
- You have the right to **appeal to the First-tier Tribunal (SEND)** about the contents of an EHCP, including provision for behaviour support.

Your rights (England)

- **SEN Support** based on needs, whether or not there is a diagnosis.
- **EHC needs assessment** on request, with statutory timelines.
- **Right of appeal** to the **First-tier Tribunal (SEND)**.
- **Right to request a particular school** in an EHCP.
- **Right to be consulted** and to have your views included.
- **Complaints** routes to the LA, the Ombudsman, and the DfE.

(In Scotland, Wales, and Northern Ireland the processes differ — ask your national advice service listed in Section 22.)

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14. Talking with your child and with other children

With your child

- Keep it simple, concrete, and brief. A 4-year-old autistic child may understand far less than their age suggests, especially in language.
- Use pictures, social stories, and role-play rather than long explanations.
- Separate the behaviour from the child: *"Hitting hurts. I love you. Let's practise safe hands."*
- Teach what **to do**, not just what not to do: *"When you want a turn, say 'my turn' or show the card."*

- Praise every approximation of the desired behaviour.
- Avoid shame. Shame increases dysregulation and aggression.

With other children (siblings, classmates)

- Age-appropriate, honest, and reassuring: *"Your brother's brain works differently. He gets overwhelmed and hits sometimes. It is not your fault, and it is not okay to hit. Here is how we stay safe."*
- Teach siblings how to get to safety and how to get an adult.
- Make sure siblings get **special one-on-one time** and are not made responsible for managing the behaviour.
- Protect siblings from injury and from being blamed.
- Consider a sibling support group (some are run by the National Autistic Society and local parent forums).

With other parents

- You may feel judged or avoid social situations. You are not alone, and you do not owe anyone a detailed explanation.
- A simple, confident script can help: *"He's autistic and gets overwhelmed easily. We're working with his nursery and therapists. Thanks for understanding."*
- Advocacy and education reduce stigma. Share as much or as little as you choose.

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15. Medication and other clinical supports

Medication is a decision for you and your child's doctor. It is **never** a first-line standalone treatment for aggression in a 4-year-old, but it can sometimes help when behavioural supports are not enough.

Discuss with your child's clinician

- **What medication is actually for:** treating the *underlying* driver — sleep, anxiety, ADHD symptoms, epilepsy, gut pain — can reduce aggression indirectly.
- **Risks and side effects**, especially for a young child, including effects on growth, sleep, appetite, and mood.
- **Trial carefully:** start low, monitor closely, keep data, and re-evaluate.
- **Never** use medication to replace teaching communication and regulation skills.
- Ask about **non-medication** options first: behavioural support (PBS), OT, SLT, sleep hygiene, and treating medical issues.

UK clinical services to pursue

- **Community paediatrician** — the usual lead for neurodevelopmental assessment and coordination.
- **Child and Adolescent Mental Health Services (CAMHS)** — for mental health, anxiety, and sometimes medication. Waits can be long; ask about the local pathway.
- **Speech and language therapy (SLT)** — communication and AAC.
- **Occupational therapy (OT)** — sensory and regulation.

- **Educational psychology** — via the setting or the LA.
- **Behaviour support services** — some areas have specialist **Positive Behaviour Support** teams, often through health, social care, or the LA. Ask your Local Offer.
- **Early years / portage services** — home-based early intervention for young children with developmental needs (ask your LA).
- **Parent training programmes** — e.g., **Incredible Years**, **Triple P**, or autism-specific parent programmes, sometimes offered free locally.

Ask each service to **coordinate and share goals** so everyone is working on the same replacement behaviours. A **Team Around the Child** or **Team Around the Family** meeting can help.

Note on ABA

Some families use **Applied Behaviour Analysis (ABA)** programmes, which are available privately in the UK and sometimes through local authority funding. Availability varies. Note that the UK community has mixed views on ABA; many families find it helpful, while some autistic advocates raise concerns. Discuss options with your child's clinicians and make sure any approach is respectful, child-led where possible, and focused on quality of life and communication.

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16. Benefits, financial help, and practical support

Aggression and high support needs can bring significant extra costs. There is help available, and it is worth claiming what you are entitled to.

Benefits (UK-wide)

- **Disability Living Allowance (DLA) for children** — for children under 16 who need extra help with care and/or mobility. It is not means-tested and does not depend on a diagnosis. Autism and significant care needs can qualify. Ask **Contact** or **Citizens Advice** for help with the form.
- **Carer's Allowance** — if you care for your child for 35+ hours a week and meet the rules.
- **Universal Credit** — may include a disabled child element (and a carer element).
- **Child Tax Credit / Working Tax Credit** — being replaced by Universal Credit, but may still apply depending on your circumstances.
- **Personal Independence Payment (PIP)** — replaces DLA from age 16.

*(Note: the benefits system is changing — Disability Living Allowance for children in Scotland is being replaced by **Child Disability Payment** via Social Security Scotland. Check your nation's current rules.)*

Other practical support

- **Short breaks / respite** — time away from caring, via the Local Offer, social care, or charities. Ask for a **carer's assessment** from your local authority (you are entitled to one).
- **Family Fund** — grants for families with disabled or seriously ill children, for equipment, breaks, and household items.

- **Local parent carer forums** — run by parents, for parents; a good source of local knowledge and support.
- **Equipment**: sensory items, weighted blankets (with OT advice), ear defenders, safety equipment.
- **SEN transport** — ask your LA if your child qualifies for home-to-school transport.

Where to get benefits advice

- **Contact** (formerly Contact a Family) — free helpline and online guides.
- **Citizens Advice** — local offices and online.
- **Turn2us** — benefits calculator and grants search.
- **Your local authority's Local Offer** — local short breaks and support.
- **Carers UK / Carers Trust** — support for carers.

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17. Tracking progress with data

You cannot tell whether a plan is working without data. Keep it simple.

What to track

- **Frequency:** how many aggressive incidents per day or week.
- **Duration:** how long each incident lasts.
- **Intensity:** severity (use a simple 1–3 or 1–5 scale) and whether anyone was hurt.
- **Triggers (A):** what preceded it.
- **Function (hypothesis).**
- **Strategy used** and **whether it worked.**
- **Replacement skills:** how often the child used words/cards instead of aggression.

Simple weekly summary table

Week	# incidents	# injuries	Most common trigger	Most common function	Replacement-skill uses	Notes
1						
2						

Rules for good data

- **Define the behaviour objectively** so everyone counts the same thing ("hit with hand or object" vs. "was aggressive").
- **Count what you want to increase too** (gentle hands, using words, calming down).
- **Keep it short.** A quick tally or app is better than a perfect log you abandon.
- **Review weekly** and share with the setting and clinicians.
- **Look for trends over 2–4 weeks**, not day-to-day. Progress is often gradual.

What progress looks like

Progress is rarely a straight line. Look for:

- Fewer incidents over time.
- Shorter, less intense incidents.
- Faster recovery.
- More use of the replacement skill.
- Fewer injuries.
- Improvement in one setting before another.

If there is **no improvement after several weeks** of a consistent plan, revisit the function hypothesis, check for medical issues again, and ask for a behaviour specialist review.

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18. Caring for yourself as a parent or carer

Your child's aggression is a heavy load. Parent and carer stress, burnout, and isolation are common and understandable — and they affect your child, so your wellbeing is part of the treatment plan, not a luxury.

Practical supports

- **Respite / short breaks** — short-term relief so you can rest. Ask your local authority for a **carer's assessment**, and check the Local Offer.
- **Support groups** — in person or online, with other parents of autistic children. Your local **parent carer forum** and the National Autistic Society can help.
- **Your own therapist** — for stress, grief, trauma, or depression. Ask your GP about talking therapies (IAPT/NHS Talking Therapies) or local counselling.
- **Practical help** — family, friends, faith communities, or paid helpers for meals, errands, or childcare.
- **Financial and legal help** — benefits, grants, and (for the future) wills, trusts, and deputyship questions. Ask **Contact**, **Mencap**, or a solicitor.
- **Sleep** — protect your own sleep where you can. Sleep deprivation makes everything harder.

Mindset shifts that help

- **Progress, not perfection.** Small gains compound.
- **The behaviour is communication, not a verdict about your parenting or your child.**
- **You are not alone.** Thousands of UK families face this, and it improves with support and time.
- **Celebrate the wins** — the calm morning, the gentle hand, the word used instead of a hit. Write them down.
- **Ask for help early and often.** Waiting until crisis makes everything harder.

Protect your relationships

- Aggression strains marriages, partnerships, and sibling relationships. Relationship counselling and family therapy can help.
- Make time, however small, for connection with your partner and other children.

19. When to seek urgent help

Get help **immediately** if:

- Someone has been seriously injured, or there is risk of serious injury.
- Your child is in danger of harming themselves or others and you cannot keep everyone safe.
- Your child has a sudden, severe change in behaviour, confusion, extreme lethargy, breathing problems, or a possible seizure.
- You feel you cannot keep yourself or others safe, or you are afraid of what you might do.
- You are in crisis and need support.

UK options:

- **999** — for immediate danger (police, ambulance, fire).
- **111** — NHS non-emergency advice, or **111 option 2** for urgent mental health support in many areas.
- **Your GP** — for urgent medical or behaviour concerns; ask about out-of-hours.
- **NHS 111 online** — 111.nhs.uk.
- **Samaritans** — **116 123** (free, 24/7) for anyone in distress, including parents.
- **Shout** — text **85258** for free 24/7 text support.
- **Your child's clinician** — community paediatrics or CAMHS, for urgent behaviour concerns.
- **Emergency social care / children's services** — if you need urgent help keeping your child safe.

Prepare a crisis plan in advance so you are not deciding in the moment:

- Who to call, in what order.
- Where to go for safety.
- What information to have ready (diagnoses, medications, triggers, what helps).
- How to keep other children safe during a crisis.

20. Templates and printable tools

A. ABC log (copy this)

Date/time	Setting	Antecedent	Behaviour (specific)	Consequence	Function guess

B. Daily setting communication log

Item	Today
Overall day (1–5)	
Number of aggressive incidents	
What triggered them	
What helped / did not help	
Replacement skill used?	
Any injury or restrictive intervention?	
What went well	
Notes for home	

C. "First-Then" / "Now and Next" and choice visuals

Create cards or a board with pictures:

- **First** [non-preferred task] → **Then** [preferred activity]
- **Now / Next** board
- **Choice board:** two pictures of acceptable options
- **Break card:** a card the child hands over to request a break
- **Feelings/needs board:** pictures for "break," "help," "more," "stop," "I want ___"

D. Calm-down steps visual

A simple sequence with pictures:

1. Stop.
2. Take a deep breath (blow out the candles).
3. Squeeze your hands.
4. Go to the cosy corner.
5. Ask for a hug or a break.

E. Social story outline

"When I Want Something"

- Sometimes I want a toy that someone else has.
- It is okay to want it.
- I can say "my turn" or show my turn card.
- I can ask a grown-up for help.
- I keep my hands down and my body safe.
- When I use my words, I can play too.

F. Emergency information card (keep in your wallet/phone)

- Child's name, age, diagnosis, and communication level.
- Medications and allergies.

- Known triggers and warning signs.
- What helps calm the child.
- Emergency contacts and clinicians' phone numbers.

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21. Glossary of terms and acronyms

Term	Meaning
AAC	Augmentative and alternative communication — tools like picture boards or speech apps
ABA	Applied Behaviour Analysis — a therapy that teaches skills and reduces problem behaviour
ABC	Antecedent–Behaviour–Consequence — a way to analyse behaviour
ALN	Additional Learning Needs (Wales)
ASD	Autism spectrum disorder
ASL / ASN	Additional Support for Learning / Additional Support Needs (Scotland)
CAMHS	Child and Adolescent Mental Health Services
CSP	Co-ordinated Support Plan (Scotland)
DLA	Disability Living Allowance
EHCP	Education, Health and Care plan (England)
EP	Educational psychologist
FCT	Functional Communication Training — teaching communication to replace problem behaviour
GP	General practitioner (family doctor)
IDP	Individual Development Plan (Wales)
IEP	Individualised Educational Programme (Scotland)
IHP	Individual Healthcare Plan
LA	Local authority
OT	Occupational therapist
PBS	Positive Behaviour Support
PECS	Picture Exchange Communication System
SEN / SEND	Special educational needs / special educational needs and disabilities
SENCO	Special Educational Needs Coordinator
SENDIASS	Special Educational Needs and Disabilities Information Advice and Support Service
SLT	Speech and language therapist
TA	Teaching assistant

22. Where to find help and reliable information

Note: Always verify current contact details and availability. The following are starting points, not endorsements.

Autism and general support (UK-wide)

- **National Autistic Society** — autism.org.uk — information, helpline, local branches.
- **Ambitious about Autism** — ambitiousaboutautism.org.uk — for children and young people.
- **Autistica** — autistica.org.uk — research and information.
- **Contact** — contact.org.uk — for families with disabled children; benefits and education advice.
- **Mencap** — mencap.org.uk — learning disability support.
- **Council for Disabled Children** — councilfordisabledchildren.org.uk.
- **Family Fund** — familyfund.org.uk — grants for families.
- **Carers UK / Carers Trust** — carersuk.org / carers.org.

Education rights and advice

- **SENDIASS** — free, impartial advice in every local authority in England (search "[your area] SENDIASS"). Statutory service.
- **IPSEA** — ipsea.org.uk — free legal advice on SEND law and EHCPs.
- **Independent Provider of Special Education Advice** — as above.
- **Contact's education advice** — contact.org.uk.
- **Enquire** — enquire.org.uk — Scotland's ASL advice service.
- **SNAP Cymru** — snapcymru.org.uk — Wales ALN advice.
- **SENAC** — senac.org.uk — Northern Ireland SEN advice.

Health

- **NHS** — nhs.uk — conditions, services, and the **NHS app**.
- **NICE** — nice.org.uk — clinical guidance (autism, constipation, sleep).
- **Royal College of Paediatrics and Child Health** — rcpch.ac.uk.

Crisis and mental health

- **999** — emergency.
- **NHS 111** — urgent medical advice; **111 option 2** for urgent mental health.
- **Samaritans** — 116 123.
- **Shout** — text 85258.
- **Mind** — mind.org.uk — mental health information.

Key questions to ask any professional

- "What is your experience with young autistic children and aggression?"
- "What is the function of this behaviour, in your assessment?"
- "What replacement skill are we teaching, and how will you measure progress?"
- "How will you coordinate with the setting and the other services?"

- "What is the plan for safety, and what do I do in a crisis?"
- "What is the local pathway and waiting time for this service?"

A closing note to the parent or carer

You are doing something very hard, and you are doing it with love. Aggression in a 4-year-old autistic child is common, understandable, and treatable. The path forward is not punishment — it is **understanding the function, keeping everyone safe, teaching better communication, and getting the right team around your child.**

Start with the quick-start steps in Section 1. Gather your data. Bring in the services. Work with the setting as a partner. And take care of yourself along the way — your child needs you regulated and supported, too.

You are not alone, and things can get better.